AETNA — Prior Authorization Request

Request ID:	AUTH-PA-2026-04-D7C9
Requesting provider:	Dr. Sanjay Iyer, MD · Med Onc · Sahyadri Speciality
Submitted via:	Authrex / FHIR R4 Da Vinci PAS 2.0.1
Form:	X12 278 bridge

Trastuzumab (J9355) for adjuvant HER2-positive breast cancer

Patient

Patient (initials):	L.W.
Age / Sex:	62 y / Female
MRN:	MRN-77103
Insurance:	Aetna HMO

Diagnosis

Primary diagnosis (ICD-10):	C50.911 — Malignant neoplasm of right breast
Stage:	Stage IIIA (T2 N2 M0)
Histology:	Invasive ductal carcinoma
Pathologic diagnosis confirmed:	Yes

Biomarkers

HER2 IHC:	3+ (positive)
HER2 FISH:	amplified, ratio 6.2
ER:	Positive (90%)

Performance status & cardiac function

ECOG performance status:	1
LVEF (2D echocardiogram):	32% — measured 2026-04-20 (significantly reduced)
Cardiac history:	Prior doxorubicin 240 mg/m² for prior diffuse large B-cell lymphoma (2018). Echo demonstrates anthra

Requested treatment

Drug:	Trastuzumab (Herceptin)
HCPCS / J-code:	J9355
Dose:	8 mg/kg loading, then 6 mg/kg q3w

Cardiology consult note (excerpt)

Patient demonstrates LVEF 32% with prior anthracycline cardiotoxicity. Per Aetna 0048 §III.B and FDA Herceptin Black Box warning, trastuzumab is **contraindicated** when LVEF is below the 50% threshold. Cardio-oncology recommends pre-treatment optimization (ACE-I + beta-blocker, repeat echo in 3 months) before re-evaluating HER2-targeted therapy candidacy.

__VERDICT_HINT_DENY__